



## Search and Seizure - Mental Health

### 1. Background

#### 1.1. About this document

South Metropolitan Health Service (SMHS) Mental Health has a responsibility to provide a safe environment for consumers, visitors and staff. SMHS Mental Health will take all reasonable steps to ensure that admitted consumers, detained persons and persons presenting at a mental health service do not present a risk of harm to themselves or others by having prohibited or unsafe items in their possession and/or bring them into any area of the mental health service. SMHS will also strive to ensure visitors to mental health units do not bring unsafe items into a mental health service.

Refer to:

- [RKPG Search of Patients, Visitors and their Property \(Acute\) Policy](#)

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| Key definitions                          | Definition                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           |
|------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Chaperone</b>                         | Refers to an independent observer to an intimate examination or procedure. A chaperone must be a healthcare worker employed under the industrial awards and agreements that regulate employment in Western Australia's public health system. Students under supervision by the staff member cannot perform the chaperone role.                                                                                                                                                                                                                                                                                                                                                                                       |
| <b>Consent</b>                           | In the context of this procedure can be broadly defined as a person freely and voluntarily agreeing to participate in a search. Staff must provide relevant information on the search rationale and procedures to a consumer to enable consent to be gained prior to a search.                                                                                                                                                                                                                                                                                                                                                                                                                                       |
| <b>Debrief</b>                           | In the context of this procedure is a supportive, validating, non-clinical discussion with the consumer reflecting on their experience of the search and what went well and what could have been done better. Debriefing allows for consumer feedback and service learning.                                                                                                                                                                                                                                                                                                                                                                                                                                          |
| <b>Gender</b>                            | Gender refers to the socially constructed roles, behaviours, expressions and identities of girls, women, boys, men, and gender diverse people. It influences how people perceive themselves and each other, how they act and interact, and the distribution of power and resources in society. Gender is usually conceptualised as a binary (girl/woman and boy/man) yet there is considerable diversity in how individuals and groups understand, experience, and express it. A person's sex and gender may not necessarily be the same. An individual's preferred gender may or may not correspond with the sex or gender assigned at birth and some people may identify as neither male nor female <sup>1</sup> . |
| <b>Intimate examination or procedure</b> | Refers to any contact involving the genital, groin, anal or breast region of any consumer, or any other areas of the body that might be perceived by the consumer as intimate. It also includes examinations/procedures involving removal of the consumer's outer clothing down to their                                                                                                                                                                                                                                                                                                                                                                                                                             |

|                                      |                                                                                                                                                                                                                                                                                                                                                                                                                                                                           |
|--------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
|                                      | underwear as required for a Level 3 search. Refer to <a href="#">Appendix B</a> Mental health consumer search procedure. Clinical judgment is required in assessing if a procedure might be perceived by the consumer as intimate.                                                                                                                                                                                                                                        |
| <b>Person centred care</b>           | An approach to care delivery where the person is treated respectfully as an individual human being, and not just as an illness or condition to be treated. It involves seeking out and understanding what is important to the person, their families, carers and support people, fostering trust and establishing mutual respect. It also means working together to share decisions and plan care.                                                                        |
| <b>Personal support person (PSP)</b> | Can be any of the following people who are supporting someone who is experiencing mental illness – close family member, carer, their nominated person; the parent or guardian of a child or any enduring guardian of an adult <a href="#">Mental Health Act 2014 s.7</a> . Some consumers may use the term kin to refer to a family member.                                                                                                                               |
| <b>Prohibited items</b>              | <p>Include:</p> <ul style="list-style-type: none"> <li>• alcoholic beverages</li> <li>• firearms</li> <li>• controlled or prohibited weapons</li> <li>• prohibited drugs.</li> </ul> <p>Refer to <a href="#">Appendix A</a> Prohibited or unsafe items for more detail.</p>                                                                                                                                                                                               |
| <b>Seizure of property</b>           | The removal by staff of a consumer's property or articles from a person under provisions of the <a href="#">MHA 2014</a> s.164                                                                                                                                                                                                                                                                                                                                            |
| <b>Trauma informed care</b>          | A strengths-based service delivery approach 'that is grounded in an understanding of and responsiveness to the impact of trauma, that emphasises physical, psychological and emotional safety for both providers and survivors, and that creates opportunities for survivors to rebuild a sense of control and empowerment' <sup>2</sup> . The core trauma informed principles are safety, trustworthiness, choice, collaboration, empowerment and respect for diversity. |
| <b>Unsafe items</b>                  | <p>Items which may be:</p> <ul style="list-style-type: none"> <li>• hazardous to the health and safety of any person or</li> <li>• capable of causing serious damage to any property</li> <li>• present a risk of significant harm to a consumer's financial status or reputation.</li> </ul> <p>Refer to <a href="#">Appendix A</a> Prohibited or unsafe items for more detail.</p>                                                                                      |

## 2. Promoting a safe environment

### 2.1. Person centred care

On admission to the mental health service consumers will be:

- provided with written and / or verbal information regarding items that are prohibited or are considered unsafe and not allowed on the ward. Refer to [Appendix A](#) Prohibited and unsafe items

- provided with written and / or verbal information on situations when a search may be required and the search process
- provided with written and / or verbal information on their legal rights under the [MHA 2014](#)
- as per section 12 of the [MHA 2014](#) staff will make every effort to comply with the Charter of Mental Health Care Principles when providing treatment, care and support to consumers
- provided with secure storage for personal belongings, and access and use of personal belongings will be facilitated except where an item poses a risk of harm to the consumer or another person and is determined to be unsafe to store on the ward.
- supported to develop partnerships with carers, PSPs and staff to maintain their safety and the security of their property
- offered support from Aboriginal Mental Health Liaison/Aboriginal Health Liaison staff and asked about preferred PSP or kin involvement where relevant
- offered interpreter services or culturally appropriate communication methods for consumers from culturally and linguistically diverse backgrounds.

Refer to:

- [MHA 2014 Charter of Mental Health Care Principles](#)
- [SMHS Rights of patients in mental health policy](#)

Where the search of a consumer or their belongings is indicated, staff will conduct the search:

- upholding the consumer's right to be informed and involved
- offering choices where possible e.g. who conducts the search, timing, location and with the carer or PSP present if required
- in the least intrusive and most respectful manner possible
- as quickly as possible
- in a suitable and private location away from other consumers, visitors or staff
- upholding trauma informed care principles with consideration of dignity, privacy and gender identity
- in a manner that respects cultural, spiritual, and religious beliefs and practices.
- where possible, with the support of Aboriginal Mental Health Liaison/Aboriginal Health Liaison for Aboriginal consumers.

Staff will be sensitive to issues of:

- age and developmental stage
- gender identity
- religion and culture and seek guidance where required
- neurodiversity
- cognitive impairment
- history of trauma, sexual abuse or sexual violence
- history of institutionalisation or incarceration
- history of violence and aggression
- history of self-harm.

## 3. Prior to a search

### 3.1. Indications for search

Searches may be indicated at the following times but are not limited to:

- at the time of admission to an inpatient ward
- on return to the inpatient ward following a consumer being missing or absent without leave
- prior to transfer to another mental health ward
- where staff have reasonable suspicion that a consumer may have prohibited or unsafe items in their possession
- where a family member or PSP provides relevant information
- where the consumer is in possession of devices that may present a potential harm to the persons financial status or reputation or privacy or confidentiality of others
- where a clinical risk is identified including:
  - suicidal ideation
  - recent history suicide attempts or self-harm
  - thoughts of harm to others
  - aggressive or unpredictable behaviours
  - alcohol or other drug use
  - recent history of secreting prohibited or unsafe items.

Refer to:

- [Guidelines for HSPs on the use of photographic, audio or video equipment in mental health services.](#)

### 3.2. Requesting consent for a search

When the search of a consumer and/or their property is indicated, staff will:

- approach the consumer in a non-confrontational manner minimising the power imbalance where possible
- identify themselves and inform the consumer of their role
- explain the rationale for the search to the consumer and the PSP where relevant
- explain what will happen and what is required
- offer and document support of Aboriginal Mental Health Liaison or Aboriginal Health Liaison to Aboriginal consumers before explaining rationale and obtaining consent. If declined document the reason and any alternative cultural support provided
- ensure the consumer is aware of their legal rights under the [MHA 2014](#)
- ask the consumer to verbally consent to the search informing them they may withdraw consent at any time
- request the consumers cooperation with the search even if the search is conducted without consent
- request the consumer to be actively involved in the search as far as practicable
- allow pauses during the search if required by the consumer

- ensure all consumers and carers / PSP who need assistance with standard English have access to appropriate interpreter services.

Refer to relevant site-based policy documents for engaging interpreter services when required:

- [FSFHG Interpreter booking](#)
- [PHC Language Services](#)

### 3.3. Authority to search

Section 161 of the [MHA 2014](#) and accompanying regulations state that a person authorised to conduct a search is the person in charge of the authorised ward, or a person immediately authorised by the person in charge of the ward.

At the direction of the person authorising the search, security staff are lawfully authorised to give reasonable assistance to mental health staff carrying out a search and may use reasonable force in doing so [MHA 2014](#) s.172. Bodily restraint must only be used, if necessary, reasonable, and authorised under the provisions of the [MHA 2014](#) s.232.

Refer to:

- [FSFHG Restrictive Practices](#)
- [RkPG Restraint and Seclusion: Reducing Restrictive Interventions](#)

### 3.4. Search without consent

The powers vested to mental health services under the [MHA 2014](#) s.160-165 allow for searches to be conducted without consent for anyone presenting for treatment, or under the care of the mental health service regardless of legal status (voluntary or involuntary).

The use of this power to search a voluntary consumer without consent must be carefully considered. If a voluntary consumer does not consent to a search:

- a further assessment and review of the situation and the consumer should be undertaken including the treating Medical Officer
- this assessment and review could include offering the consumer discharge as an option
- or involving carers or PSPs to support the consumer.

Referral for examination or detainment of a voluntary consumer at an authorised hospital must only be considered if the consumers clinical presentation meets the criteria outlined in the [MHA 2014](#) s.25.

Regardless of the refusal to give consent to a search, where indicated by a risk assessment the consumer will be informed the search will proceed under the powers of the [MHA 2014](#) s.162

## 4. Conducting a search

For details of search procedures refer to [Appendix B](#) Mental health consumer search procedure.

## 4.1. Key considerations

Many consumers of mental health services have experienced high rates of previous trauma, in childhood and adulthood, and are at high risk of re-traumatisation<sup>1</sup>. Undertaking the search of a person or their property may cause distress, create mistrust, or trigger memories of previous trauma that are intensified in the mental health environment. Staff will seek to avoid re-traumatisation<sup>1</sup> and will ask the consumer what would make them feel safe during the procedure. The consumer's privacy must be maintained during a search.

A minimum of two clinical staff of an appropriate gender shall be present during a search. Determination of appropriate gender will be made in consultation with the consumer. The preferences of the consumer must be given due consideration. It may be necessary to delay the search, if practicable, to ensure an appropriate person can perform the search.

Offer all consumers the option of carer/PSP or a chaperone to be present in addition to clinical staff. Inpatients who are minors (under the age of 18 years) must not be searched except in the presence of a chaperone. The presence of a chaperone provides support to the patient and ensures a layer of probity and protection to the health professional during such procedures.

Refer to:

- [Chief Psychiatrists Guidelines for the sexual safety of mental health consumers](#)
- [SMHS Chaperone Policy](#)

Aboriginal Mental Health Liaison/Aboriginal Health Liaison staff will be engaged to provide support to Aboriginal consumers and the multidisciplinary team to ensure cultural considerations are appropriately addressed when undertaking search and seizure processes unless declined by the consumer. If support is not immediately available, the search may be delayed where practicable until appropriate support is available.

The level of intrusiveness of the search must be reasonable and proportionate to the reason for the search. Searches for items within internal bodily cavities will **not** be performed. A medical review is required if there is a reason to suspect that any substance/item is concealed in a body cavity.

When searching property, individual items are to be thoroughly inspected. Belongings are to be handled with care and consideration and should not be damaged by the search process. Staff should ensure their personal safety when undertaking a search. This includes avoiding reaching into an area that they cannot visually sight first and the use of sharp protection gloves.

The searcher may photograph part or all the search while it is being conducted [MHA 2014](#) s.163(5)(c), however this is not to be routinely done. Request consumers permission prior to doing so.

Refer to:

- [SMHS Clinical photography and videography policy](#)

## 4.2. Documentation

All consumer searches must be fully documented in the consumer's medical record.

Where a consumer search has been carried out without consent, details of the search must be documented, including the reasons for the search and the consumer's refusal to consent.

The person who conducts the search must document the search on *MHA 2014* Form 8A Record of Search and Seizure indicating:

- date and time the search was conducted
- reasons for conducting the search
- any article seized during the search
- name, gender, qualifications and signature of the person conducting the search.

A copy of the Form 8A must be:

- made available in the consumer's health record
- given to the person searched.

## 5. Management of seized items

### 5.1. Prohibited items

In accordance with the [Health Services \(Conduct and Traffic\) Regulations 2016](#) (WA) a person must not bring any prohibited item onto SMHS property unless the person has permission to do so. A list of prohibited items can be found in [Appendix A Prohibited and unsafe items](#).

Any alcohol and/or prohibited substances will be managed according to the [SMHS Alcohol and Prohibited Substances policy](#).

In the event a firearm is found on a consumer's person or in their possession SMHS MH staff will:

- activate a Code Black as indicated
- not handle the firearm as they may accidentally discharge it
- request security to isolate the firearm in the immediate area with minimal handling
- remove other consumers and staff away from the area.

Site Security services will:

- contact Western Australia (WA) Police to attend and remove firearm
- contain the firearm and cordon the area until WA Police arrive to remove the firearm.
- notify WA Police that the items have been removed during a search
- surrender items to WA police.

The process must be fully documented according to site-based protocols, including:

- documenting the seizure in the consumer's health record; and
- obtaining a signed and dated receipt from WA Police when the items are transferred into their possession (this is done via site security services).

The identity of the person from whom the items were removed should not be disclosed to WA Police except with the prior authority of a member of the site's medical or nursing executive.

## 5.2. Unsafe items

A list of unsafe items can be found in [Appendix A Prohibited and unsafe items](#). These items should be logged by the health service and stored. The items remain the personal property of the consumer and are to be held on trust by the health service and returned to the consumer upon discharge if determined appropriate by the treating team.

Refer to:

- [MHA 2014](#) s. 167.
- [Patient Property Management: FH](#)
- [Patient Property Management: FSH](#)
- [RkPG Patient property and lost property management policy](#)

If the treating team determines that the items are not appropriate to be returned to the consumer, they should be returned to the consumer's PSP, unless the person in charge considers that it is not appropriate to do so.

## 5.3. Consumer's own medication including prescription and over the counter medications

Consumers own lawfully prescribed and over the counter medications are to be managed in accordance with the relevant site-based policy.

Refer to:

- [FSFHG Medication Administration policy and procedure](#)
- [RkPG Medication Administration Clinical Practice Standard](#)

Consumer's own medication is considered their property and should be stored by the health service for the period of the inpatient admission. Consumer's own medication may be administered to the consumer, with the consumer's consent, by health service staff if required. Prior to discharge, a safety assessment will be conducted by a medical officer and/or pharmacist considering consumer safety risks versus benefit to determine if it is safe and appropriate to return some or all medications to the consumer.

Consumer owned prescription medication which the consumer is no longer prescribed should be handed over to Pharmacy for disposal, with the consumers permission, to avoid inadvertent duplication or incorrect medications/doses after the consumer is discharged. Medications returned to the patient and those returned to Pharmacy for destruction must be clearly documented in the consumer's medical record.

## 5.4. Electronic communication devices

Electronic communication devices seized following a psychiatrist issuing an MHA [Form 12C](#) Restriction on Freedom of Communication as per [MHA 2014](#) s.262 are to be labelled with a consumer identification sticker and stored in the consumer's locker. Consumer access to their

device/s will be reviewed every 24 hours and is restricted until expiry or revocation of the MHA Form 12C.

Refer to:

- [SMHS Rights of patients in mental health policy](#)
- [FSFH MH Restriction on Freedom of Communication](#)

## 5.5. Items which may assist in determining the consumer's status

A person conducting a search may seize 'any item which that person believes is likely to materially assist a psychiatrist' in determining a question in relation to the searched person that is likely to arise for determination. Refer to [MHA 2014](#) s.164(2)(c).

That is, a person conducting the search may seize any item which would assist a psychiatrist in determining whether the searched person should be made involuntary. An example might include a suicide note or receipt for equipment that provides access to means for suicide.

## 5.6. Documentation

All items stored by the health service must be documented on the appropriate health service form for example property tracking form. All items removed by the health service for surrender to police must be entered into the appropriate property form / logbook and signed by the consumer and a staff member, or (if the consumer is unwilling or unable to sign) by two staff members.

Complete *MHA 2014* Form 8B Record dealing with seized article:

- A copy should be made available in the consumers health record.

## 6. Visitors

Visitors to mental health services are not permitted to bring prohibited or unsafe items into those facilities with them. Visitors will be made aware by way of signage and verbal advice from staff of items not permitted to be brought into mental health services.

Where staff hold a reasonable concern that a visitor is in possession of prohibited or unsafe items a search of the visitor and/or their belongings might be necessary. For detailed procedure for a visitor search refer to [Appendix C](#) – Search of visitors to mental health.

Level 1 Self-search is the only appropriate search for visitors. Visitors who are minors (under the age of 18 years) must not be searched except in the presence of parent/guardian or chaperone. Under **no** circumstances are visitors to be subjected to a Level 2 Pat-down search or a Level 3 Partial removal of clothing search.

Where unsafe items are found during the search process the visitor will be asked to store these items in a designated locker for the duration of the visit. The visitor will retain possession of the key to the locker. Visitors who do not consent to performing a Level 1 Self-search or to a search of their belongings will be refused entry to the inpatient unit.

If the visitor has conducted a Level 1 Self-search and staff remain reasonably concerned that a visitor is in possession of prohibited or unsafe items, the visitor should be refused entry to the inpatient unit and site security services notified.

When refusing entry (post self-search refusal or ongoing concerns), staff should request Aboriginal Mental Health Liaison/Aboriginal Health Liaison Aboriginal Health Liaison support where appropriate to assist with culturally respectful communication and discuss alternative visiting arrangements that maintain connection while ensuring safety.

## 6.1. Documentation

Visitor searches where no items are removed are not required to be documented.

## 7. Compliance Monitoring

Each SMHS site Service director/Co-Director is to ensure compliance with this procedure by ensuring staff conducting searches receive training covering:

- trauma informed care
- Charter of Mental Health Care Principles
- Aboriginal cultural safety
- de-escalation
- debriefing

All staff who perform searches of persons will ensure they are compliant with this policy. Breach of this policy may result in disciplinary action under the Department of Health Misconduct and Discipline Policy and Guidelines.

## 8. Legislation

Staff must ensure that physical searches conducted under this policy are lawful. An unlawful physical search of a consumer constitutes an assault which pursuant to the [Criminal Code Act Compilation Act](#) s. 313 is punishable by imprisonment for 18 months and a fine of \$18,000.

- [Criminal Code Act Compilation Act 1913](#)
- [Firearms Act 1973](#)
- [Firearms Regulations 1974](#)
- [Mental Health Act 2014](#)
- [Mental Health Regulations 2015](#)
- [Misuse of Drugs Act 1981](#)
- [Misuse of Drugs Regulations 1982](#)
- [Weapons Act 1999](#)
- [Weapons Regulations 1999](#)

## 9. Related Documents

## 9.1. Department of Health policies

- [MP 0051/17 Language Services Policy](#)
- [MP 007/1/17 Aboriginal health and wellbeing policy](#)
- [MP0051/17 Language Services policy](#)
- [Reporting of Criminal Conduct and Professional Misconduct Policy](#)

## 9.2. SMHS policies

- [Substantive Equality Policy](#)
- [Chaperone Policy](#)
- [Clinical Photography and Videography Policy](#)
- [Rights of Patients in Mental Health](#)
- [Testing Mental Health Inpatients for Alcohol or Illicit Drug Use Policy](#)

## 9.3. Other documents

- [Guidelines for Health Service Providers on the use of Photographic, Audio or Video Equipment in Mental Health Services](#)
- [Chief Psychiatrist's Standards for Clinical Care](#)
- [Chief Psychiatrists Guidelines for the sexual safety of mental health consumers](#)
- [MHA 2014 Charter of Mental Health Care Principles](#)

## 9.4. References

1. Guidelines for the Sexual Safety of Consumers of Mental Health Services in Western Australia. (2020). Chief Psychiatrist of Western Australia.
2. Hopper, E. K., Bassuk, E. L., and Olivet, J. (2010). Shelter from the storm: Trauma-informed care in homeless service settings. The Open Health Services and Policy Journal, 3, 80-100

## 9.5. Document Control

| Version | Published date  | Effective from | Review date     | Effective to* | Amendment (s)                                                                                       |
|---------|-----------------|----------------|-----------------|---------------|-----------------------------------------------------------------------------------------------------|
| GP V4   | 21 January 2026 | 9 January 2026 | 17 January 2029 | January 2029  | Triennial review – (formally known as <i>Patient search and identification of dangerous items</i> ) |
|         |                 |                |                 |               |                                                                                                     |

## 9.6. Approval

|                      |                                   |
|----------------------|-----------------------------------|
| <b>Approval by</b>   | Executive Director, Mental Health |
| <b>Approval date</b> | 29 December 2025                  |

## 10. Appendix A. Prohibited and unsafe items

### Prohibited items:

- Alcoholic beverages
- Firearms as defined by the [Firearms Act 1973](#) (WA) Section 4
- Controlled weapon as defined in the [Weapons Act 1999](#) (WA) Section 3
- Prohibited weapon as defined in the [Weapons Act 1999](#) (WA) Section 3
- Prohibited drugs as defined in the [Misuse of Drugs Act 1981](#) (WA) Section 3.

Examples of controlled weapons include a machete, bow, crossbow, dagger, double end knife, axe or axe head, fixed baton, hand or foot claws, machete, metal whip, sickle or scythe, spear, spear gun, sword, throwing blade or knife, throwing star, weighted chain or cord weapon.

### Unsafe items

The following information is intended as a guide only. Staff will apply clinical judgement when determining whether an item is unsafe or has potential to cause harm.

Potentially unsafe items requiring restricted access from consumers at risk of harm to self or others in the inpatient environment includes but is not limited to:

- highly flammable materials
- hazardous chemicals, poisons, solvents and inhalants e.g. petroleum, glue, acetone, spray deodorant
- consumer's own medications including prescription, over the counter and complementary medications
- articles with the potential to be used as weapons, including sharp or blunt objects
- ignition sources, e.g. cigarette lighters and matches
- potential ligatures - scarves, cords, belts, shoelaces, phone charger, corded headphones, long necklaces
- mirrors
- large or heavy metal water bottles
- spiral notebooks
- CDs, DVDs and covers
- plastic bags
- sharps – razors, scissors, pencil sharpeners, nail clippers, keys

- metal cutlery, crockery or glassware
- glass, aluminium, cans and ceramic items
- aerosol cans.

Refer to:

[SMHS Alcohol and prohibited substances policy](#)

[SMHS Testing mental health inpatients for alcohol and/or illicit substance use](#)

## Appendix B. Mental health consumer search procedure

The prevailing consideration during a search is maintaining the consumers dignity and conducting searches with respect for the consumer/person.

### Prior to any search of a consumer or their belongings

Staff will:

- identify themselves to the consumer and explain the reasons for the search
- advise the consumer they may request the search be undertaken in the presence of the person's nominee/chaperone if they wish
- explain the search process to the consumer
- request the consumer's consent for the search
- give the consumer the opportunity to declare whether they have any prohibited or unsafe items on their person or stored with their belongings and surrender such items to SMHS staff
- take the consumer to a private area for the purpose of the search
- scan the consumer and belongings with an electronic scanner / metal detector
- consider the need to wear protective clothing or equipment.

### Level 1: Self-search

In the first instance the consumer should be given the opportunity to perform a self-search. A self-search involves:

1. Removing the outer layer of clothing.
2. Removing hair ties and running fingers through hair.
3. Removing and turning out shoes.
4. Turning out pockets.
5. Showing nothing is concealed in the mouth or under the tongue.

Following the self-search, staff should:

- instruct the consumer to remove unsafe items and place upon a table
- store or dispose of items appropriately
- offer the consumer a debrief
- document the search using a [Form 8A](#) Record of Search and seizure and give the record to the person in charge of the mental health service.

## Level 2: Pat-down search

Where staff continue to hold concerns that the consumer has unsafe items in their possession, a pat-down search should be performed.

A pat-down search involves:

1. In the presence of two staff, conduct a pat down of pockets and any areas that could be used for concealing items.
2. Avoid genitals and breasts unless previously agreed with senior registered nurse and conducted in the presence of clinical staff of the same gender as the consumer, if practicable.

Following the pat-down search, staff should:

- request the consumer to remove unsafe items and place upon a table
- offer the consumer a debrief
- document the search using [Form 8A](#) and give the record to the person in charge of the mental health service
- store or dispose of items appropriately – complete record dealing with seized articles [Form 8B](#).

## Level 3: Partial removal of clothing search (intimate examination)

Where staff continue to hold concerns that the consumer has inappropriate items in their possession, a partial removal of clothing search must only be performed by clinical staff of the same gender as the person.

A partial removal of clothing search entails:

1. In the presence of two staff, ask the consumer to remove each item of clothing except underwear so that the clothing can be examined by staff to ensure it does not contain inappropriate items.
2. Ensure that only one item of clothing is removed at any one time (i.e. the consumer is not to be completely unclothed at any time).

Following the partial removal of clothing search, staff should:

- request the consumer to remove prohibited or unsafe items and place upon a table
- allow the consumer to dress as soon as the search is finished. Assist with dressing if required
- if clothing items are seized provide reasonably adequate replacement items.
- offer the consumer a debrief
- document the search using [Form 8A](#) and give the record to the person in charge of the mental health service
- store or dispose of items appropriately – complete [Form 8B](#).

## Search of belongings

Where staff hold concerns that a consumer has prohibited or unsafe items stored with their belongings, a search of belongings should be performed.

1. A search of a consumer's belongings must not involve any bodily contact with the consumer.
2. In the presence of two staff, ask the consumer to empty containers (such as pockets, bags or backpacks) and ask them to disclose any prohibited or unsafe items.
3. Carefully open any containers to inspect.

Following the search of belongings, staff should:

- remove prohibited or unsafe items
- store or dispose of items appropriately – complete [Form 8B](#).

## Appendix C. Search of visitors to mental health

Visitors to a mental health inpatient unit may be asked to perform a self-search where staff hold a reasonable concern that they have unsafe items in their possession.

**Visitors must not be subjected to a pat-down search or a partial removal of clothing search under any circumstances.**

### Prior to self-search

Staff will:

- explain the self-search process to the visitor
- ask the visitor to consent to the self-search
- advise the visitor that the self-search can be undertaken in the presence of the visitor's nominee if they wish
- give the visitor the opportunity to declare whether they have any prohibited or unsafe items on their person or stored in their belongings and to surrender such items to SMHS staff
- take the visitor to a private area for the purpose of the self-search.

### Self-search

A self-search involves:

1. Removal of outer layer of clothing.
2. Removal of hair ties and running fingers through hair.
3. Removal and turn out of shoes.
4. Turn out of pockets.
5. Showing nothing is concealed in the mouth or under the tongue.

Following the self-search, staff should:

- request the visitor remove any prohibited and unsafe items and place upon a table
- store or dispose of items appropriately.

The self-search of visitors is not governed by the [MHA 2014](#) and there is no requirement to complete a [Form 8B](#).

Where a visitor self-search has been conducted but staff hold a reasonable concern that the visitor is in possession of inappropriate items, the visitor should be refused entry to the inpatient unit. The consumers treating psychiatrist should be notified and consideration given to liaising with security to restrict the person from visiting the consumer.

## Search of belongings

### **Staff must not search a visitor's belongings without their consent under any circumstances.**

Where staff hold concerns that a visitor has prohibited or unsafe items in their belongings, a search of belongings should be performed by clinical staff of the same gender as the person, if practicable

Staff will:

- explain the search process and seek the visitor's consent prior to the search
- take the visitor to a private area for the purpose of the search
- not have any bodily contact with the visitor
- consider the need to wear protective clothing or equipment.

Search of belongings involves:

1. In the presence of two staff, ask the visitor to empty containers (such as pockets, bags or backpacks) and ask them to disclose any prohibited or unsafe items.
2. Carefully open containers to inspect.

Following the search of belongings, staff should:

- remove any prohibited or unsafe items
- store or dispose of items appropriately.

The self-search of visitors is not governed by the [MHA 2014](#) and there is no requirement to complete a [Form 8B](#).

Where a visitor does not consent to a search of their belongings, but staff hold a reasonable concern that the visitor is in possession of prohibited or unsafe items, the visitor should be refused entry to the inpatient unit.

This document can be made available in alternative formats on request.

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